

Institutionalization by another route

What Alberta's Bill 53 does, what the evidence says, what it costs, and how it fits a continental turn toward confinement.

A campaign breakdown. Every load-bearing figure and legal claim in this document was checked against a primary source in August 2026. Where a number is a calculation or an estimate rather than a published figure, it says so. Where a policy is a proposal rather than an enacted law, it says so.

This is not legal advice. It describes what the documented record establishes, in plain terms.

The Alberta Disability System Breakdown

St. Albert, Alberta | August 2026

1. What Bill 53 does

Bill 53, the **Compassionate Intervention Act** (SA 2025 c C-21.5), was sponsored by Dan Williams, Minister of Mental Health and Addiction, in Premier Danielle Smith's UCP government. It received first reading on April 15, 2025, third reading on May 13, 2025, and Royal Assent on May 15, 2025. Third reading carried on a **voice vote**, with no recorded division, so there is no truthful member-by-member list of who voted for it.

The Act does more than authorize locked treatment. It builds a supervised relationship that follows a person out of the building. The powers below are confirmed against the enacted text:

Secure compulsory treatment	A secure care plan order allows up to three months of treatment in a secure compassionate intervention facility (ss.48 to 49). To apprehend means to detain a person without consent (s.1(b)).
Community care orders	A community-based care plan order runs up to six months (ss.1, 48). Designated supervisors oversee the person in the community and non-compliance is reviewed (ss.13, 54).
Police apprehension	Police may apprehend, detain and convey a person, and re-apprehend, using reasonable force (s.34 and related sections).
No right to refuse	A client does not have the right to refuse to be observed, monitored and assessed by a treatment team (s.80(2)(a)).
Treatment without consent	A client cannot refuse a drug administered as authorized by the Commission, and a regulated professional may provide treatment without consent, including administering a drug (s.80(2)(c), s.80(3)).
Information without consent	The director and the Commission may indirectly collect individually identifying health and personal information without the person's consent (s.87(2)).
Disclosure to family and police	Identifying health and personal information may be disclosed without the client's consent to family members (s.87(3)) and, in the apprehension context, to police (s.87(4)).
A standing commission	The Compassionate Intervention Commission is established (s.7(1)), with regulation-making power to add reporting, oversight and monitoring requirements (ss.13 to 15, 92, 93).

The objection: an institution without walls

A person can leave the locked building and still be inside the institution: monitored treatment, mandatory reporting, police enforcement, information sharing, and re-confinement for non-compliance.

Because the factors that draw a person into the system, such as difficulty meeting daily needs, emergency-department use, relationship breakdown and effects on community safety, are shaped by poverty and homelessness, the poor and the publicly visible are far more exposed than affluent people who use substances privately.

2. The evidence for compulsory treatment is weak

The Canadian Centre on Substance Use and Addiction (CCSA), in its February 2025 evidence brief on involuntary treatment for severe substance use disorders, found the evidence limited and methodologically weak. In its own words, most studies do not show significant improvements in reducing substance use or reoffence; involuntary treatment has been linked to negative outcomes including the possibility of increased mortality following discharge; and the programs are comparatively resource-intensive and costly. The brief concludes that if used at all, it should be viewed as a last resort. (Scope note: the CCSA brief addresses civil involuntary treatment, not court-ordered treatment.)

CAMH, the Centre for Addiction and Mental Health, reached an unusually direct conclusion in its September 2025 policy brief on Ontario's corrections proposal: involuntary substance use treatment in the corrections system **is not the answer**. CAMH instead recommends building voluntary treatment and supportive housing, opioid agonist therapy, equitable access for people in corrections, and continuity of care after discharge.

The core problem is that confinement can produce short-term abstinence without durable recovery. Unless housing, income, medication, relationships and continuing treatment exist after release, the person returns to substantially the same circumstances, now with reduced drug tolerance and a heightened overdose risk.

A fair concession

Short emergency detention can sometimes prevent an immediate death or serious injury. That does not establish that months of compulsory rehabilitation, correctional treatment or renewable confinement is effective. The two claims should not be traded for one another.

3. Confinement can undermine treatment itself

Effective treatment ordinarily depends on trust, truthful disclosure and a working therapeutic relationship. When disclosure can trigger apprehension, confinement or a compliance report, the incentives invert:

- people may avoid emergency departments, outreach workers and voluntary programs;
- patients learn that disclosure can be used against them;
- clinicians take on policing and reporting roles;
- resistance to coercion can be read as further evidence of illness or non-compliance; and
- restraint, medication and segregation become the tools that enforce care.

The CCSA brief records patient experiences of fear, trauma, aggression, intimidation, privacy violations and separation from children. CAMH warns that forced medication may involve restraint, create choking and other safety risks, provoke responsive aggression, and lead to segregation for a failure to participate.

4. The money: confinement versus community care

Alberta's Budget 2026 provides **\$291 million over three years (2026 to 2029)** to build two 150-bed compassionate intervention centres in Edmonton and Calgary, about 300 beds, opening around 2030, operated by Recovery Alberta. Budget 2026 also lists **\$33 million in 2026** for compassionate intervention operations, and the government is preparing **about 100 interim secure beds** in existing hospitals and facilities. (The original Budget 2025 figure was \$180 million; what rose between budgets is the capital cost, not the bed count.)

Two numbers to label plainly

About \$970,000 in capital per planned bed is a campaign calculation (\$291 million divided by 300 beds), not a figure the government published. It is capital only, before a single nurse, doctor, guard, administrator or treatment worker is paid.

There is no published mature operating cost per occupied bed. Illustrative arithmetic from the current budget (capital amortized over 30 years plus the \$33 million operating line) points to a locked bed plausibly costing on the order of \$142,000 or more per year. Treat that as an estimate; the final government figure is undisclosed.

Against that, the evidence on community care is concrete. The Mental Health Commission of Canada's At Home / Chez Soi Housing First trial found that every \$10 invested returned service-cost offsets of \$9.60 for high-needs participants, \$3.42 for moderate-needs participants, and \$21.72 among the highest-cost 10 percent, mostly through reduced use of hospitals, shelters and crisis services. Housing First was not housing only: participants kept receiving community treatment and support. In Calgary, Housing First has cost roughly \$14,000 to \$30,000 per person per year, with about \$2 in public-system savings for every \$1 spent (Jadidzadeh, Falvo and Dutton, 2020).

Ordinary housing with community support	about \$14,000 to \$30,000 per person per year (Calgary Housing First)
Older Canadian institutional estimates	about \$66,000 to \$120,000 per person per year
Alberta locked model, illustrative from the current budget	about \$142,000 or more per bed per year (estimate; government figure undisclosed)

The construction cost alone, annualized over 30 years, is larger than the full annual cost of housing and supporting someone through Housing First. That is the comparison the government has not answered: confinement adds custody, security, institutional capital and legal administration on top of care, and the state should show those added costs buy better outcomes before it moves money away from voluntary services that already cannot meet demand.

The scale of unmet voluntary demand is documented. In Ontario, at least **36,378 people** were waiting for mental health or addiction supportive housing, with an average wait of about four years (Addictions and Mental Health Ontario, 2025, cited by CAMH). Building locked capacity while voluntary housing and treatment remain inaccessible reverses the normal order: the state will fund care after removing a person's

liberty, but not always while they are asking for it.

What \$291 million could purchase instead

At Calgary's documented Housing First cost, \$291 million represents roughly 9,700 to 20,800 person-years of housing and support. At a midpoint near \$20,000 a year, it could house 300 people for approximately 48 years. That comparison is not inflation-adjusted, and it excludes the locked model's future operating, security and maintenance costs, which would make the gap wider, not narrower.

A community alternative built on existing stock

Statistics Canada's 2021 Census recorded **139,450** Alberta dwellings not occupied by usual residents (the province's unoccupied rate was 7.6 percent). Not all are usable: after removing seasonal, under-renovation, on-market and temporarily occupied homes, a smaller pool of truly available dwellings remains. That netted figure is an estimate, not a Statistics Canada count, and should always be labelled as such. Even so, to find 300 homes the province would need a very small fraction of the available stock. A community model could:

- identify habitable vacant dwellings near services and suitable for long-term residence;
- offer owners guaranteed government leases, damage coverage and professional management, and purchase persistently vacant properties where leasing is uneconomic;
- renovate publicly owned or tax-delinquent dwellings;
- provide rent-geared-to-income leases directly to unhoused people, with mobile community teams for voluntary mental health, addiction, medical and daily-living support;
- keep housing unconditional, so losing treatment participation would not mean losing the home; and
- apply a vacancy tax to owners who keep habitable homes empty while refusing occupancy or the leasing program.

5. The civil and disability rights that are engaged

These laws are not automatically unconstitutional because they are coercive; a court must weigh each provision and its safeguards. But several Charter rights are plainly engaged, and unwanted medical treatment is recognized by Justice Canada as an interference with bodily integrity under section 7:

Section 7	life, liberty, bodily autonomy and security of the person
Section 9	protection against arbitrary detention
Section 10	reasons for detention, access to counsel, and habeas corpus
Section 12	protection against cruel or grossly disproportionate treatment, in extreme cases
Section 15	equal protection and benefit of the law without discrimination based on mental or physical disability

Canada has also ratified the Convention on the Rights of Persons with Disabilities, whose provisions include equal legal capacity, liberty, bodily integrity, informed consent and the right to live in the community. In its 2025 review of Canada, the UN disability committee raised concern that people are still forced into institutions because community support is unavailable. The UN's deinstitutionalization guidance warns that

adding beds, replacing large institutions with smaller ones, or renaming institutions does not by itself satisfy the right to community living.

6. The political map: this crosses party lines

Enacted or operational programs are separated from proposals below, because the difference matters. This list is drawn from enacted legislation, government announcements and explicit public statements as verified in August 2026. It is not every politician in Canada: some bills passed on a voice vote, and silence is not endorsement.

Enacted or operational

Alberta (UCP)	Bill 53, the Compassionate Intervention Act. Secure and community compulsory treatment, police apprehension, wide information-sharing. Passed on a voice vote, so no member-by-member list exists. Danielle Smith; sponsor Dan Williams.
Saskatchewan (Sask. Party)	Bill 48, the Compassionate Intervention Act, passed third reading May 5, 2026 (in force expected fall 2026). Minister Lori Carr championed it; Justice Minister Tim McLeod defended the legal framework; the Scott Moe government passed it. The Saskatchewan NDP opposed.
British Columbia (BC NDP)	Highly secure involuntary-care facilities under the Mental Health Act, long-term secure care (Monarch Homes at Alouette Correctional), more than 400 involuntary mental-health beds, and a secure unit at Surrey Pretrial. Announced September 15, 2024 by David Eby and Jennifer Whiteside; beds opened in 2025 and expanded in 2026 under Josie Osborne and Terry Yung.
Manitoba (NDP)	Bill 48, the Protective Detention and Care of Intoxicated Persons Act, allowing detention of highly intoxicated people for up to 72 hours. This is protective detention and stabilization, not renewable compulsory rehabilitation, so it sits in an adjacent category. Wab Kinew government; passed with near-unanimous support.

Announced, proposed or promoted, not enacted

British Columbia (BC Conservatives)	John Rustad campaigned in 2024 for a broader involuntary-treatment expansion. Both major BC parties therefore endorsed forms of compulsory care. Proposal.
Ontario (PC)	The Ford government moved from studying toward exploring compulsory treatment for people in jail, on probation or parole, looking more to BC's model. Associate Minister Michael Tibollo's verified position is that voluntary capacity must be built first and the idea is worth studying. Ontario has not enacted a civil-commitment law.
New Brunswick (former PC)	The Higgs government proposed a Compassionate Intervention Act with forced treatment (Kris Austin, Sherry Wilson). It was shelved before the 2024 election and never enacted.

Federal (Conservative)

Pierre Poilievre promised in the 2025 campaign to let judges order mandatory treatment for certain non-violent offenders and require rehabilitation in prisons. Proposal.

Who should not be on a supporter list

Alberta's NDP opposed Bill 53 at final reading. Saskatchewan's NDP opposed Bill 48. BC Greens criticized the involuntary-care expansion. Ontario NDP, Liberal and Green representatives criticized the encampment and notwithstanding-clause proposal. Federal Health Minister Marjorie Michel, a Liberal, said there is no scientific evidence that forced addiction treatment works, and declined to intervene against provincial plans. Non-intervention is not endorsement.

Municipal endorsements exist too. BC's 2024 secure-care announcement drew supportive statements from Vancouver Mayor Ken Sim, Maple Ridge Mayor Dan Ruimy, and local First Nations leaders. In late October 2024, thirteen Ontario mayors (Barrie, Brampton, Brantford, Cambridge, Oakville, Oshawa, Pickering, St. Catharines, Greater Sudbury, Guelph, Chatham-Kent, Clarington and Windsor) wrote to Premier Ford seeking stronger mandatory-treatment powers for people in encampments and, where necessary, use of the notwithstanding clause. That letter was a separate initiative: the Ontario Big City Mayors' collective resolution did not endorse the notwithstanding clause.

7. The Canada and United States connection

Two neighbouring countries with the world's longest border, integrated economies, shared media and professional networks are developing strikingly similar policies inside the same two-year window. Treating that as coincidence unless someone produces a signed coordination agreement is an artificially weak standard. Coordination is only one possible mechanism. Policy convergence, legal borrowing, shared narratives and political imitation do not require a written agreement.

The fair framing is this: **direct coordination is not established. A tightly timed, structurally consistent continental convergence is.** The United States has made the direction explicit while Canada builds the operational counterpart.

The timeline

September 2024	BC announces highly secure involuntary facilities, correctional treatment and hundreds of involuntary psychiatric beds.
April to May 2025	Alberta introduces and passes Bill 53: secure compulsory treatment, community orders, police apprehension and surveillance.
July 24, 2025	US Executive Order 14321, 'Ending Crime and Disorder on America's Streets,' directs the use of civil commitment to shift homeless individuals into 'long-term institutional settings,' seeks reversal of precedents and consent decrees that impede commitment, and calls for 'maximally flexible' civil-commitment and institutional treatment standards. Section 2 is titled 'Restoring Civil Commitment.'
November 2025	Manitoba passes extended protective detention (up to 72 hours).

May 5, 2026	Saskatchewan passes Bill 48, the Compassionate Intervention Act.
June 18, 2026	A US Department of Justice Office of Legal Counsel opinion concludes that neither Section 504 nor Title II of the ADA imposes an integration mandate, and that neither authorizes agencies to require community-based care. This is an internal executive-branch legal interpretation, not a rule or a court ruling.
July 20, 2026	The DOJ states it will not rely on its Olmstead enforcement guidance and treats it as not enforceable (Federal Register notice).
Late January 2026	In Texas v. Kennedy (the renamed continuation of Texas v. Becerra, filed September 2024), states file an amended complaint challenging the HHS Section 504 integration-mandate regulation, not Section 504 itself.
2026	Alberta transfers AISH recipients into the new Alberta Disability Assistance Program (ADAP).
2026 to 2030	Alberta prepares about 100 interim secure beds and commits \$291 million to build about 300 permanent beds.

Canada began operationalizing parts of the structure before the US order; the US now leads in explicitness, federal scale and legal doctrine. Trump's order states openly what Canadian governments generally place under softer language: the intended destination is long-term institutional placement, and the legal barriers to civil commitment should be removed. Canada calls it compassionate intervention, secure care and protective detention. The American order calls it institutionalization.

How the two directions reinforce one another

Ideological legitimacy. Once a large country defines institutionalization as humane and as public-order restoration, confinement reads as a modern option rather than a historical regression, and governments can move without saying they are reopening institutions.

Legal development. Canadian courts are not bound by US decisions, but American disability arguments are read and adapted here: that community placement is not required by statutory text, that governments must keep discretion over treatment settings, that community services impose excessive costs, and that courts should defer on resource allocation.

Political normalization. When the same policy appears across the continent, each government makes the others look less exceptional. Each borrows the others' language and moves the boundary of what the public will accept.

8. Where the NDP fits, and why ADAP matters

This context makes the opposition's record more serious, not less. If institutionalization were only a UCP project, the NDP's opposition to Bill 53 might reassure. But NDP governments are already participants: the BC NDP is building secure involuntary capacity, and the Manitoba NDP extended protective detention. The Alberta NDP is not standing outside an alien ideology; it belongs to a political family whose governments have already accepted parts of it.

Asked directly how the party would abolish ADAP, Alberta's NDP social-services critic answered that she did not know, that she is not the leader, and that there is no clear plan. In the middle of a continental turn toward confinement, a party seeking control of Alberta's completed machinery has no stated plan to dismantle it. An incoming government could inherit the whole apparatus: ADAP's classification structure, medical reassessment, employment-service contractors, Bill 53, the Compassionate Intervention Commission, community treatment orders, information-sharing powers, the interim secure beds and the two permanent facilities. It could restore the \$200, improve procedural safeguards, and run the rest under kinder language. That is substantially what NDP governments in BC and Manitoba demonstrate: coercive state power can be retained and rebranded as progressive care.

9. The argument, in short

This is not a set of isolated provincial experiments. It is a North American movement developing across a shared border, on a compressed timeline, through similar language, similar target populations and similar powers. The United States is making it explicit through an executive order, a Department of Justice opinion, and litigation aimed at the integration mandate. Canada is building the operational counterpart: locked beds, police apprehension, compulsory treatment, correctional care, community monitoring and disability classification.

The objection is not that emergency intervention can never be justified. It is that governments are investing in confinement before making voluntary treatment, supportive housing, income, medication and community assistance reliably available. The evidence for long-term compulsory treatment is limited and mixed, while the costs, the loss of liberty, the surveillance, the trauma and the post-release overdose risk are concrete. When a person can obtain housing and treatment only after the state has authorized police apprehension and removed the right to leave, that is not an adequate community-care system. It is institutionalization by another route.

The precise claim

We have not established the exact channels of any coordination, and this document does not assert them. What is established is a highly proximate, tightly timed and structurally consistent continental convergence that is too substantial to frame as a collection of isolated coincidences.

10. How this was verified

Every load-bearing claim above was checked against a primary source in August 2026. The main sources:

Bill 53 text and status	Compassionate Intervention Act, SA 2025 c C-21.5; Legislative Assembly of Alberta Bill 53 text and Bill Status Report (first reading Apr 15, third reading May 13 on a voice vote, Royal Assent May 15, 2025).
Alberta costs	alberta.ca/compassionate-intervention and Budget 2026 highlights (\$291M over 2026 to 2029; \$33M in 2026; about 100 interim secure beds; centres open around 2030).

Evidence	CCSA evidence brief, Involuntary Treatment for Severe Substance Use Disorders (February 2025); CAMH policy brief (September 16, 2025).
Housing economics	Mental Health Commission of Canada, At Home / Chez Soi National Final Report (2014); Jadidzadeh, Falvo and Dutton, Canadian Public Policy (2020); Addictions and Mental Health Ontario (2025) for the 36,378 waitlist.
Dwellings	Statistics Canada 2021 Census, Alberta: 139,450 dwellings not occupied by usual residents; unoccupied rate 7.6 percent. The smaller 'truly available' figure is an advocacy estimate, not a Statistics Canada count.
Other jurisdictions	Saskatchewan Bill 48 (passed May 5, 2026); BC government release, Sept 15, 2024; Manitoba Bill 48; Ontario reporting; New Brunswick reporting; Poilievre 2025 campaign; Minister Michel, June 2025; the 13-mayor letter, October 2024.
United States	Executive Order 14321 (July 24, 2025), Federal Register 90 FR 35817; DOJ Office of Legal Counsel opinion (June 18, 2026); DOJ Federal Register notice, 91 FR 45287 (July 20, 2026); Texas v. Kennedy, N.D. Tex. 5:24-cv-00225 (renamed from Texas v. Becerra), amended complaint late January 2026.

Three honesty notes carried through the document: figures marked as a calculation (for example about \$970,000 per bed) are the campaign's own arithmetic, not government figures; the smaller vacant-dwelling figure is an estimate, not a census count; and proposals (BC Conservatives, Ontario, New Brunswick, the federal Conservatives) are kept separate from enacted laws (Alberta, Saskatchewan, British Columbia, Manitoba).

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